

--SUMMARY--

Decision No. 3769/17

26-Mar-2018

K.Cooper

- Consequences of injury
- Second Injury and Enhancement Fund {SIEF} (severity of accident)
- Second Injury and Enhancement Fund {SIEF} (severity of preexisting condition)

No Summary Available

24 Pages

References: Act Citation

- WSIA

Other Case Reference

- [w2218n]

Style of Cause:

Neutral Citation: 2018 ONWSIAT 1013



WORKPLACE SAFETY AND INSURANCE APPEALS TRIBUNAL

DECISION NO. 3769/17

BEFORE:

K. Cooper: Vice-Chair

HEARING:

December 4, 2017 at Toronto
Oral

DATE OF DECISION:

March 26, 2018

NEUTRAL CITATION:

2018 ONWSIAT 1013

DECISION(S) UNDER APPEAL: WSIB Appeals Resolution Officer (ARO) decision dated
October 14, 2015

APPEARANCES:

For the worker:

A. Langan, Lawyer

For the employer:

C. Stewart, Lawyer

Interpreter:

N/A

Workplace Safety and Insurance
Appeals Tribunal

505 University Avenue 7th Floor
Toronto ON M5G 2P2

Tribunal d'appel de la sécurité professionnelle
et de l'assurance contre les accidents du travail

505, avenue University, 7^e étage
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REASONS**(i) Introduction**

- [1] The employer appeals a decision of the ARO, which concluded that the worker had ongoing entitlement for a right knee strain, right hand abrasion, and a right medial meniscus tear, as well as to a right lateral meniscus tear. The ARO also concluded that the worker was entitled to Loss of Earnings (LOE) benefits resulting from right knee surgery on October 4, 2013 covering a period from October 4 to October 13, 2013. The ARO also determined that the employer was entitled to 50% SIEF based upon a moderate accident imposed over a moderate pre-existing condition.
- [2] On May 24, 2013 this Personal Support Worker (PSW) was leaving the building when she slipped on wet leaves/seed pods, lost her balance, turned and twisted her right knee outward, and put out her right hand to break her fall.
- [3] The Eligibility Adjudicator's June 4, 2013 decision accepted initial entitlement and noted the accepted diagnoses were right knee strain, right leg torn soleus muscle and right hand abrasions. The claim was allowed for health care benefits only, as the employer offered suitable modified work to begin on May 29, 2013 at no wage loss.
- [4] The worker resumed modified duties on May 31, 2013 and objected to the June 4, 2013 decision providing further medical information for consideration. The Eligibility Adjudicator's June 26, 2013 reconsideration noted the worker had sought medical attention on May 28, 2013 and was authorized off work for two days. The lost time was accepted and Loss of Earnings benefits were extended for the two days lost time.
- [5] The worker made ongoing complaints of pain and was referred to the WSIB Surgical Specialty Program. The Case Manager's September 5, 2013 decision noted the results of the July 17, 2013 MRI confirmed a flap tear involving the anterior horn of the lateral meniscus and a probable displaced meniscal tear fragment. Mild medial and lateral compartment osteoarthritis were also noted. Surgery was recommended through the specialty program and approved; however, there was no entitlement to the osteoarthritic changes as these were considered pre-existing.
- [6] The worker underwent surgical repair on October 4, 2014. The worker lost time from work and was paid LOE benefits from October 4, 2013 to October 13, 2013.
- [7] The employer requested SIEF relief of costs. The Case Manager's June 11, 2014 decision confirmed while osteoarthritis was present, it did not prolong or enhance the recovery or lost time, and SIEF was denied.
- [8] The employer objected to the denial of SIEF and the worker's ongoing impairment. The Case Manager's September 4, 2014 decision confirmed ongoing entitlement was established through the reports from the Specialty Program at the Toronto Western Hospital confirming ongoing right knee symptoms.
- [9] The ARO's decision giving rise to this appeal upheld the operating level decisions with the exception that the employer was awarded 50% SIEF.
- [10] The employer objected to some of the ARO findings leading to the appeal herein.

(ii) Issues

[11] The issues under appeal are as follows:

1. Entitlement for a right lateral meniscus tear and post-traumatic osteoarthritis.
2. Ongoing entitlement to the right knee, including the surgery of October 4, 2013.
3. Entitlement to LOE benefits from date of surgery to October 13, 2013.
4. SIEF relief of 90% based on a severe pre-existing condition.

(iii) Background**(a) Forms**

[12] An Employer's Form 7, dated May 31, 2013 recorded injuries to the worker's right hand and knee, with the accident mechanism described as:

Employee states she slipped on seed pods that were scattered on the walkway that lead [sic] to the entrance of facility, and as a result she lost her balance and as she started to fall, she felt her right knee turn outward causing immediate pain to knee. Employee put out right hand to break her fall resulting in abrasions to right hand.

[13] The Worker's Form 6, dated June 7, 2013 noted injuries to her right hand, knee and ankle. It described the mechanism of injury as:

Coming out of the building, slipped on wet leaves on the way to my car.

(b) Board memoranda

[14] Memo #1, dated May 31, 2013 set out the initial contact with the worker. It noted the following accident mechanism:

F7 wr slipped on seed pods that were scattered on the walkway leading to the entrance of the facility. Wr lost her balance and started to fall. Wr felt her right knee turn outward causing immediate pain. Wr broke her fall with her right hand resulting in abrasions to her right hand.

[15] Memo #3, dated June 2, 2013 recorded a statement from the worker which noted:

24May2013 the wr was leaving [employer's premises] after seeing a client, exited the building into the rain. Stepped on wet leaves and seeds. slipped and fell. The wr's right knee went one way while she was falling and her body went another way. The wr experienced immediate right knee pain and also scraped her right hand on the ground.

The wr did not think that her injury was very serious at the time

27May2013 the wr called her family gp and was able to book an appointment for 28May2013. 28May2013 the wr was seen by Dr. Thomas diagnosed her with a right MCL strain and a torn soleus muscle.

[16] Memo #33, dated September 5, 2013 contained the decision to allow eligibility for FLAP tear and surgery to correct same. It also noted no eligibility for OA as "condition was present prior to the date of accident and not caused by the accident."

[17] Memo #110, dated June 10, 2014 set out the CM's review of the Employer's claim for SIEF. The memo noted:

Entitlement was accepted for a right knee strain and tears that occurred on October 4, 2013 when the worker leaving a building and she slipped on wet leaves and seeds and fell causing her knee to go one way with her body going the other way. The worker felt immediate pain to the knee and scraped her hand on the ground. Based on this information, noting the added peril of the wet leaves and seeds on the ground, and the unexpected nature of the slip and fall, I am accepting the accident history is of a moderate severity as it would reasonably be expected to cause a disabling injury

PRE-EXISTING CONDITION:

I have reviewed the information on file including the MRI dated July 16, 2013 that indicates there is mild medial and lateral compartment osteoarthritis. There is a superiorly surfacing flap tear involving the body and anterior horn of the lateral meniscus. There is also some irregularity to the inner margin of the posterior horn of the lateral meniscus in keeping with associated minor inner margin tearing. There is a 4 mm structure posterior to the central sweep of the posterior horn of the medial meniscus with similar signal intensity as the menisci. Probably a displaced meniscal tear fragment. There is a small Baker's cyst. The operative report dated October 4, 2013 was reviewed and gives a pre and post operative diagnosis of torn right medial meniscus, torn right lateral meniscus and osteoarthritis right knee. The reports do not indicate there are any pre-existing factors delaying the worker's recovery

DURATION:

Loss of earnings benefits were paid from May 29, 2013 to May 31, 2013 and from October 4, 2013 to October 15, 2013.

RECOMMENDATION:

Based on the above information, I am unable to grant entitlement to SIEF. This worker's claim was allowed on its own merit that is the direct result of a work accident. The surgery was accepted and the worker continues in treatment at present. Although there are findings of osteoarthritis, this is an incidental finding as there is no indication it enhanced the injury or prolonged the recovery.

[18] Memo #145, dated November 17, 2014 contains a review and opinion of Board Medical Consultant (BMC) Dr. Gelman. He noted:

[The worker] is a 49 year old woman who injured her right knee in May, 2013. This resulted in a meniscal tears and posttraumatic arthritis.

[19] Memo #174, dated June 3, 2015 contained a reconsideration by the CM. It noted:

CM's review of claim supports entitlement to both tears to the right knee medial meniscus and the lateral meniscus.

Worker had twisting knee injury when she slipped and fell onto the ground. I concur with Dr. Marshall as per his report of 12jan15 that tears of the medial and lateral related to the fall at work.

Furthermore there is no evidence worker at a symptomatic right knee injury prior to her fall, given the complex tear of the lateral meniscus.

The surgery performed on 04Oct13 to repair these tears are in order as is the resulting lost time.

Entitlement confirmed for the lateral and medial meniscus and the surgeries lost time paid from 29may13 to 31may13 in order as is LOE from 04Oct13 to 15Oct13. Worker returned to modified work/hours 15Oct13 gradual return to work receiving full advances from her employer.

[20] Memo #188, dated November 23, 2015 set out the consideration of a permanent impairment (PI) for the worker. It noted:

On 24MAY2013, [the worker] suffered injuries to her right knee, right lower leg and right hand when she slipped on some wet debris, twisted her knee and fell. She suffered some abrasions to her right hand. The original diagnosis was a right MCL strain and torn soleus muscle.

[The worker] did undergo prior right knee surgery in 2003 calendar year and had some pre-existing osteoarthritis in her knee at time of work accident.

Medical Mitigations:

[The worker] started receiving physiotherapy treatment as of 26JUN2013, see Physiotherapist report on file See MRI of right knee dated 16JUL2013.

See Specialty clinic report dated 21AUG2013, arthroscopic surgery for right knee arranged. See operative report dated 04OCT2013.

[The worker] continued with physiotherapy treatment post-surgery See Physiotherapist report dated 07OCT2013.

See Specialty clinic reports dated 18OCT2013 to 26MAY2014. Slow recovery from surgery noted. [The worker] experienced swelling in right calf. Assessment for possible DVT arranged. Pain consultation arranged for possible chronic regional pain syndrome (CRPS). Corticosteroid injections arranged. Specialist consultation arranged to review need for possible total knee replacement.

See Specialty clinic reports dated 17JUN2014 to 12JAN2015. Further MRI of right knee arranged, see results dated 05JUL2014. Total knee replacement not recommended. PRP injections or further arthroscopy recommended, this treatment was not approved for WSIB coverage. Further recovery not expected without proposed treatment.

See m#187, [the worker] has advised me that she has not received any significant treatment since January 2015 and none is planned.

MMR Date:

Appears that [the worker] reached maximum medical recovery (MMR) for work-related injuries as of Specialty clinic assessment on 12JAN2015.

PI Recommendation:

Permanent work-related impairment appears evident for [the worker's] right knee with diagnosis of torn right medial and lateral meniscus and post-traumatic arthritis.

(c) Medical reports

[21] An MRI of the right knee dated July 16, 2013 found:

There is mild medial and lateral compartment osteoarthritis.

No abnormal joint effusion. There is a small Baker's cyst.

The cruciate and collateral ligaments are intact.

There is a superiorly surfacing flap tear involving the body and anterior horn of the lateral meniscus. There is also some irregularity to the inner margin of the posterior horn of the lateral meniscus in keeping with associated minor inner margin tearing.

There is a 4mm structure posterior to the central sweep of the posterior horn of the medial meniscus, with similar signal intensity as the menisci. This is probably a displaced meniscal tear fragment. This may arise from the lateral meniscus, or potentially could arise from the central sweep of the posterior horn inner margin, and be posteriorly flipped.

[22] A report dated August 21, 2013 from Dr. Marshall, an orthopaedic specialist, stated:

Torn right lateral meniscus.

Torn right medial meniscus.

In summary, this woman suffered a rotational injury to her right knee and has MRI documented medial and lateral meniscal tears with displaced fragment. She has not returned to her pre-injury level of function and clearly requires arthroscopic intervention. We have made her aware of the risks and benefits and she has decided she would like to proceed.

Overall Prognosis

Current Status: She is partially recovered. Further recovery will not occur in the absence of arthroscopic intervention.

[23] A report dated February 24, 2014 from Dr. Marshall noted:

I am concerned that she may require reconstructive surgery for her advanced lateral compartment osteoarthritis. If she fails to get improvement with her most recent injection, I would recommend assessment by an arthroplasty surgeon after her next visit.

[24] A report dated February 24, 2014 from Dr. Bhatia, a pain specialist, noted:

The last MRI, which was performed on 16 July 2013, revealed evidence of early-to-moderate osteoarthritis in the right knee along with tear of the medial and lateral menisci. An MRI has not been repeated since the arthroscopy.

[25] A report dated March 24, 2014 from Dr. Marshall noted:

Her recovery has been slow and she continues to have significant pain and significant underlying osteoarthritis. It is too soon to render a long-term prognosis.

[26] A report dated May 26, 2014 from Dr. Marshall noted:

I would recommend a specialty consultation with Dr. Nizar Mahomed with a view towards potentially proceeding with a right total knee arthroplasty. It should be noted that she had full thickness loss of articular cartilage on her lateral femoral condyle at time of arthroscopy.

[27] A report dated June 17, 2014 from Dr. Mahomed, an orthopaedic surgeon, noted:

This 49-year-old lady works as a personal support worker and had an injury to her right knee on May 24, 2013. This is a work-related injury where she was leaving the Nursing Home, slipped on a wet surface and had a twisting injury to her knee. She has a past history of problems with that knee and had arthroscopic surgery to that knee in 2001.

...

Arthroscopic findings at the time of surgery by Dr. Marshall would suggest that she has advanced lateral compartment OA, although this is not evidenced by her pain imaging.

[28] An MRI dated July 5, 2014 noted:

Conclusion: Postsurgical changes of the medial meniscus. Combination of postsurgical changes of the lateral meniscus with an area of undersurface tearing of the anterior horn of the lateral meniscus. Severe chondrosis of the mid to posterior aspect of the lateral femoral condyle with milder changes involving the posterior aspect of the lateral tibial plateau. Mild patellofemoral chondrosis. Joint effusion with synovitis.

[29] A report dated July 29, 2014 from Dr. Mohamed noted:

Overall, the MRI confirms that she has had her previous meniscal tear treated with arthroscopic surgery and she has some element of osteoarthritis in this knee; however, the extent of arthritis in her knee is not severe enough that she would be a candidate for total knee arthroplasty, nor would she, therefore, be benefiting from knee replacement surgery.

[30] A report dated August 11, 2014 from Dr. Marshall noted:

It should be noted that her surgical findings are in keeping with her having had a degree of pre-existing injury to the lateral compartment, but this was significantly exacerbated by the compensable traumatic injury, which resulted in a lateral meniscal tear and significant loss of articular cartilage over the lateral femoral condyle. She had no pre-existing symptoms prior to her injury.

[31] A report dated October 20, 2014 from Dr. Marshall noted:

It should be noted that prior to her work -related injury, she was completely asymptomatic. The meniscal tear and posttraumatic arthritic changes are entirely in keeping with her injury. Although she has recovered from the meniscal symptoms, she continues to have significantly disabling posttraumatic arthritic symptoms, most notably in the lateral compartment. In the absence of proceeding with PRP injections, it is likely that she will require arthroscopy and microfracture of her lateral femoral condyle.

[32] A report dated January 15, 2015 from Dr. Marshall noted:

To clarify the findings at surgery, the subject had had a previous arthroscopy of her right knee in 2003. Subsequent to the arthroscopy she had been asymptomatic until her accident of May 24, 2013. At the time of her accident she had a slip and fall in which she violently rotated her right knee and landed on the lateral aspect. At time of arthroscopy, we found a complex tear of her lateral meniscus as well as full thickness loss of articular cartilage on the lateral femoral condyle and to a lesser extent the lateral tibial plateau. There was no degenerative change in the patellofemoral joint and very minimal change in the medial compartment. There was a small degenerative medial meniscal tear. Thus, although there may have been an element of some relatively mild asymptomatic degree of pre-existing osteoarthritis in the right knee, the findings at surgery of a large lateral meniscal tear and full thickness loss of articular cartilage and lateral femoral condyle were fully in keeping with the degree of violence that she suffered in the slip and fall accident of May 24, 2013. Thus, her continuing problems in my view are predominantly if not entirely related to the slip and fall accident of May 24, 2013.

[33] An extensive report dated March 18, 2015 from Dr. Offierski, an orthopaedic surgeon, was requested by counsel for the accident employer. Dr. Offierski noted at the outset of his review that it was a paper review of a 39-year old worker who had “slipped on some leaves and seed pods and twisted her right knee.” He also noted that the information on file indicated a “worker sustaining a twisting injury to the knee when she slipped and twisted the knee. The symptoms arising from this injury were confined to the medial portion of the knee only. The worker did not think the injury was serious.” Following a recitation of his understanding of the medical history of the worker, he opined:

It is my opinion that the ongoing pain, swelling and disability in the right knee arc due to the pre- existing arthritic condition of the lateral compartment of the right knee. The worker had previous arthroscopic surgery to the right knee fifteen years prior. It has been noted in the file that the worker has had problems with weight control over the years. This would account for the presence of the pre- existing arthritic condition in the lateral compartment of the knee. It has been concluded that the ongoing pain and disability the worker experiences is due to the arthritic condition of the knee, which is unrelated to the work injury of May 24, 2013 rather than the torn medial meniscus.

I would disagree with the SIEF decision that states the reports do not indicate any pre-existing factors delaying the worker's recovery. The MRI scan and the operative findings document significant arthritic deterioration in the lateral compartment of the knee which has been determined to be the cause of the worker's ongoing disability.

In summary, it is my opinion that the worker sustained a mild to moderate twisting injury to the right knee that could have torn the medial meniscus and required the arthroscopy and meniscectomy. The expected outcome of the above-noted surgery was that the worker would achieve relief of pain and disability 8 weeks after the surgery.

(d) NEL award

[34] The worker's NEL award of a 14% whole person impairment was made on the basis of entitlement to right knee medial and lateral meniscal tears, and post traumatic arthritis. The award granted a 15% impairment for the right knee on the basis of "Arthritis due to any etiology, including trauma." The award went on to note that the ARO decision giving rise to the appeal herein indicated that the arthritis was a moderate pre-existing condition, and as such the award was being reduced by 10% for the lower extremity.

(iv) Testimony

[35] The worker testified that she was 49 years old at the time of the accident, and had started with the employer in 2007 as a PSW.

[36] She stated that on the date of accident it was raining and as she was walking her foot slipped on leaves lying on the ground, and she fell forward with both knees hitting the ground. She stated that she had put her right hand out to break her fall. She stated that she slid approximately two feet with her right knee splayed out alongside her.

[37] The worker testified that she then went home and noticed she had "road rash" on her bilateral knees and right hand. She stated that her bilateral knees and ankles had started to swell. The worker stated that she had bruising of her right leg and left knee, as well as a mark on her face.

[38] The worker stated that she did not seek medical assistance immediately as it was the weekend and she had hoped it would get better over the course of her two days off. She stated that on Monday, however, her right knee was swollen. The worker stated that she then put on a brace and went to work. She stated that she went to see her doctor the following day, and also informed her employer about the incident. The worker stated that she did not think the injury was that serious at the time.

[39] The worker stated that in 2001 she had suffered from "water on the knee," which was drained, and that she had no further issues with her right knee until the workplace accident. She stated that in the years leading up to the accident her right knee was asymptomatic. The worker testified that she had always been able to perform her regular duties, and that she had not required any time off or accommodations for the right knee prior to the 2013 accident.

[40] The worker stated that she had undergone a gastric bypass in 2008, after which she lost over 130 pounds. The worker stated that she had never been diabetic, although her doctors had stated she was pre-diabetic just before her gastric bypass surgery.

[41] The worker stated that she had never met, nor been examined by, Dr. Offierski.

[42] The worker stated that following the accident she was in considerable pain, which eventually led to the right knee surgery at issue. She stated that the surgery did not help much. The worker stated that prior to the surgery she felt the greatest pain in the front part of her knee, and after the surgery the pain was all around her knee. The worker stated that since the surgery she had undergone various treatment regimens with varying, but ultimately unsatisfying, results.

[43] Under cross-questioning from Ms. Stewart, counsel for the employer, the worker restated the accident, noting that as she was falling forward her right leg twisted, with her right toe pointing inward. She stated that the right knee bore the brunt of the fall, and that her left knee did not swell.

[44] The worker stated that the “brace” she wore to work on the Monday was actually a tensor bandage belonging to her husband.

[45] The worker stated that she did not think the slip and fall was that serious, and that she did not think at the outset that she would require medical treatment.

[46] The worker testified that she had arthroscopic surgery on her right knee in 2001 to drain fluid from her knee, and that she had blocked the Board’s request to see her medical records from that time as there was nothing relevant to be found in them.

[47] The worker stated that when reporting the incident to her employer, she had not mentioned her right ankle as the main issue was her knee.

[48] The worker stated that she was not considered a diabetic as she was able to control her blood sugars with diet.

(v) Law and policy

[49] Since the worker claimed to be injured in 2013, the *Workplace Safety and Insurance Act*, 1997 (the WSIA) is applicable to this appeal. All statutory references in this decision are to the WSIA, as amended, unless otherwise stated.

[50] An “accident” is defined in section 2(1) to include:

- (a) a wilful and intentional act, not being the act of the worker,
- (b) a chance event occasioned by a physical or natural cause, and
- (c) disablement arising out of and in the course of employment;

[51] General entitlement to benefits is governed by section 13:

13(1) A worker who sustains a personal injury by accident arising out of and in the course of his or her employment is entitled to benefits under the insurance plan.

(2) If the accident arises out of the worker’s employment, it is presumed to have occurred in the course of the employment unless the contrary is shown. If it occurs in the course of the worker’s employment, it is presumed to have arisen out of the employment unless the contrary is shown.

[52] The statutory presumption set out in section 13(2) does not apply to an injury by disablement. See, for example, *Decisions No. 268* and *42/89*.

[53] Tribunal jurisprudence applies the test of significant contribution to questions of causation. A significant contributing factor is one of considerable effect or importance. It need not be the sole contributing factor. See, for example, *Decision No. 280*.

- [54] The standard of proof in workers' compensation proceedings is the balance of probabilities. Pursuant to subsection 124(2) of the WSIA, the benefit of the doubt is resolved in favour of the claimant where it is impracticable to decide an issue because the evidence for and against the issue is approximately equal in weight.
- [55] OPM Document No. 15-02-01, "Definition of Accident," describes a chance event as "an identifiable unintended event which causes an injury," an injury itself is not a chance event. The policy defines a disablement as "a condition that emerges gradually over time" or "an unexpected result of working duties."
- [56] OPM Document No. 11-01-01, "Adjudicative Process," states that an allowable claim must have five points: an employer, a worker, personal work-related injury, proof of accident, and compatibility of diagnosis to accident history. OPM Document No. 11-01-01 provides the following guidelines for determining proof of accident:

Proof of accident

Decision-makers may consider the following when examining proof of accident,

- Does an accident or disablement situation exist?
- Are there any witnesses?
- Are there discrepancies in the date of accident and the date the worker stopped working?
- Was there any delay in the onset of symptoms or in seeking health care attention?

- [57] *Operational Policy Manual* (OPM) Document No. 14-05-03, entitled "Second Injury and Enhancement Fund" was included in the Case Record. This policy provides in part:

Policy

If a prior disability caused or contributed to the compensable accident, or if the period resulting from an accident becomes prolonged or enhanced due to a pre-existing condition, all or part of the compensation and health care costs may be transferred from the accident employer in Schedule 1 to the SIEF.

Both physical and psychological disabilities are included.

Guidelines

There is no provision in the Act for the Fund to apply to Schedule II employers.

In situations where alcoholism plays a role in the causation of an accident, it is not considered to be a pre-existing condition with regard to the application of SIEF relief.

The objectives of this policy are to provide employers with financial relief when a pre-existing condition enhances or prolongs a work-related disability. It thereby encourages employers to hire workers with disabilities.

Definitions

Pre-accident disability is defined as a condition which has produced periods of disability in the past requiring treatment and disrupting employment.

Pre-existing condition is defined as an underlying or asymptomatic condition which only becomes manifest post-accident.

(...)

SIEF-application to employer costs

Medical significance of pre-existing condition*	Severity of accident**	Percentage of cost transfer***
Minor	Minor Moderate Major	50% 25% 0%
Moderate	Minor Moderate Major	75% 50% 25%
Major	Minor Moderate Major	90%-100% 75% 50%

NOTES

* The medical significance of a condition is assessed in terms of the extent that it makes the worker liable to develop a disability of greater severity than a normal person. An associated pre-accident disability may not exist.

With psychological conditions, the possibility of prior psychic trauma resulting from life experience could be considered as evidence of vulnerability, and justify recommending relief to the employer, even in the absence of pre-existing psychological impairment.

** The severity of the accident is evaluated in terms of the accident history and approved definitions.

Accident History Components

- mechanics (lift, push, pull, fall, blow, etc.)
- position (kneeling, standing, sitting, squatting, bending, etc.)
- environment (lighting, temperature, weather conditions, terrain, etc.)

(vi) Submissions

[58] Ms. Stewart, counsel for the employer, submitted that the worker should not have entitlement for osteoarthritis, noting that the CM had made this finding as found in the Case Record. She also submitted that the medical literature does not support the development of post-traumatic OA in the brief period of time elapsed between the worker's accident and this diagnosis. Ms. Stewart further submitted that the worker should not be entitled to the lateral tear in her right knee due to the advanced and pre-existing OA found in that compartment. She also submitted that Dr. Marshall showed bias in his reporting on behalf of the worker, and thus less weight should be attached to his opinions. She also submitted that the medical evidence on file supported that the worker's pre-existing condition was "severe." She submitted that the worker's post-traumatic OA was more likely a result of the worker's 2001 right knee arthroscopy. Ms. Stewart submitted that the medial tear was compensable, and had recovered following surgery, but that the non-compensable lateral tear was continuing to give the worker issues. She submitted that in the medial compartment the worker's pre-existing condition was moderate, and that should the lateral tear also be allowed than SIEF relief in the amount of 90% would be in order. Ms. Stewart also submitted various Tribunal decisions in support of her arguments.

[59] Ms. Langan, counsel for the worker, submitted that the medical evidence does not support a severe pre-existing condition, and that at most the worker had a minor pre-existing condition which was then seriously aggravated by the accident. She stated that the type of fall in

the wet conditions of the day led to the accident being classified as moderate in nature. Ms. Langan also submitted various Tribunal decisions in support of her arguments.

[60] In reply, Ms. Stewart submitted that there had been inconsistencies in the reporting of the accident mechanism, with the first mention of direct trauma to the knees being in 2015.

(vii) Analysis

(a) Entitlement to a right knee lateral meniscus tear, ongoing right knee entitlement, entitlement to right knee surgery, and LOE from date of surgery to October 13, 2013.

[61] In this matter, the worker has been awarded entitlement to a right lateral meniscus tear, and ongoing right knee entitlement, both of which are objected to by the accident employer. The employer has indicated that they accept entitlement for a right knee strain, right hand abrasion, and medial meniscus tear. I will examine the issue of post traumatic OA of the right knee, which is also partially objected to by the employer, below.

[62] The employer has argued that the majority of the worker's ongoing right knee issues relate to her lateral meniscus tear (and pre-existing OA), and that this tear is not the result of the workplace accident, but rather sequelae to the non-compensable and pre-existing OA.

[63] The Board allowed entitlement to the right lateral meniscus tear as noted in Memo #174, dated June 3, 2015:

CM's review of claim supports entitlement to both tears to the right knee medial meniscus and the lateral meniscus.

Worker had twisting knee injury when she slipped and fell onto the ground. I concur with Dr. Marshall as per his report of 12Jan15 that tears of the medial and lateral related to the fall at work.

Furthermore there is no evidence worker at a symptomatic right knee injury prior to her fall, given the complex tear of the lateral meniscus.

The surgery performed on 04Oct13 to repair these tears are in order as is the resulting lost time.

Entitlement confirmed for the lateral and medial meniscus and the surgeries lost time paid from 29May13 to 31May13 in order as is LOE from 04Oct13 to 15Oct13. Worker returned to modified work/hours 15Oct13 gradual return to work receiving full advances from her employer.

[64] As also noted by the ARO, prior to the workplace injury the worker's right knee was found to be asymptomatic, and that following the injury it never returned to its pre-accident state. The ARO also noted that in the absence of any symptomatic findings or pre-accident impairment there was no limitation on ongoing entitlement for the right knee. The "thin skull principle," found in common law, is simply the doctrine of taking the person as they are found to be at the time in question. Thus, although the worker may have had a pre-existing condition that resulted in a greater injury than may be expected from the accident itself, this does not limit entitlement where the condition was asymptomatic prior to the accident as in this case. Put another way, but for the accident the worker may have continued to work without complaint or symptoms for many years.

[65] There is a five point checklist (set out above) that is used to determine initial entitlement. In this case we have an employer, a worker, an accident, and an injury, but the compatibility of the lateral meniscus tear to the accident is in dispute.

[66] The accident was described at the outset of the claim as such:

- An Employer's Form 7, dated May 31, 2013 which recorded injuries to the worker's right hand and knee, with the accident mechanism described as:

Employee states she slipped on seed pods that were scattered on the walkway that lead [sic] to the entrance of facility, and as a result she lost her balance and as she started to fall, she felt her right knee turn outward causing immediate pain to knee. Employee put out right hand to break her fall resulting in abrasions to right hand.

- Memo #1, dated May 31, 2013 which set out the initial contact with the worker. It noted the following accident mechanism:

F7 wr slipped on seed pods that were scattered on the walkway leading to the entrance of the facility. Wr lost her balance and started to fall. Wr felt her right knee turn outward causing immediate pain. WR broke her fall with her right hand resulting in abrasions to her right hand.

- Memo #3, dated June 2, 2013 which recorded a statement from the worker which noted:

24May2013 the wr was leaving [employer's premises] after seeing a client, exited the building into the rain. Stepped on wet leaves and seeds. Slipped and fell. The wr's right knee went one way while she was falling and her body went another way. The wr experienced immediate right knee pain and also scraped her right hand on the ground.

[67] This accident mechanism appears to be the one most contemporaneous to the event, and the one used to evaluate her injury going forward by various doctors and other health professionals. The worker indicated at the hearing, and in 2015 to the Board, that she had landed on both knees and slid two feet following the fall. I prefer the contemporaneous documentary evidence, together with an absence of evidence of significance supporting an injury to the left knee and a two foot slide.

[68] Based on the aforementioned mechanism of injury, the following medical evidence was produced:

- An MRI of the right knee dated July 16, 2013 which found:

There is mild medial and lateral compartment osteoarthritis.

No abnormal joint effusion. There is a small Baker's cyst.

The cruciate and collateral ligaments are intact.

There is a superiorly surfacing flap tear involving the body and anterior horn of the lateral meniscus. There is also some irregularity to the inner margin of the posterior horn of the lateral meniscus in keeping with associated minor inner margin tearing.

There is a 4mm structure posterior to the central sweep of the posterior horn of the medial meniscus, with similar signal intensity as the menisci. This is probably a displaced meniscal tear fragment. This may arise from the lateral meniscus, or potentially could arise from the central sweep of the posterior horn inner margin, and be posteriorly flipped.

- A report dated August 21, 2013 from Dr. Marshall, an orthopaedic specialist, which stated:
Torn right lateral meniscus.

Torn right medial meniscus.

In summary, this woman suffered a rotational injury to her right knee and has MRI documented medial and lateral meniscal tears with displaced fragment. She has not returned to her pre-injury level of function and clearly requires arthroscopic intervention. We have made her aware of the risks and benefits and she has decided she would like to proceed.

Overall Prognosis

Current Status: She is partially recovered. Further recovery will not occur in the absence of arthroscopic intervention.

- A report dated August 11, 2014 from Dr. Marshall which noted:

It should be noted that her surgical findings are in keeping with her having had a degree of pre-existing injury to the lateral compartment, but this was significantly exacerbated by the compensable traumatic injury, which resulted in a lateral meniscal tear and significant loss of articular cartilage over the lateral femoral condyle. She had no pre-existing symptoms prior to her injury.

- A report dated October 20, 2014 from Dr. Marshall which noted:

It should be noted that prior to her work -related injury, she was completely asymptomatic. The meniscal tear and posttraumatic arthritic changes are entirely in keeping with her injury. Although she has recovered from the meniscal symptoms, she continues to have significantly disabling posttraumatic arthritic symptoms, most notably in the lateral compartment. In the absence of proceeding with PRP injections, it is likely that she will require arthroscopy and microfracture of her lateral femoral condyle.

- A report dated January 15, 2015 from Dr. Marshall which noted:

To clarify the findings at surgery, the subject had had a previous arthroscopy of her right knee in 2003. Subsequent to the arthroscopy she had been asymptomatic until her accident of May 24, 2013. At the time of her accident she had a slip and fall in which she violently rotated her right knee and landed on the lateral aspect. At time of arthroscopy, we found a complex tear of her lateral meniscus as well as full thickness loss of articular cartilage on the lateral femoral condyle and to a lesser extent the lateral tibial plateau. There was no degenerative change in the patellofemoral joint and very minimal change in the medial compartment. There was a small degenerative medial meniscal tear. Thus, although there may have been an element of some relatively mild asymptomatic degree of pre-existing osteoarthritis in the right knee, the findings at surgery of a large lateral meniscal tear and full thickness loss of articular cartilage and lateral femoral condyle were fully in keeping with the degree of violence that she suffered in the slip and fall accident of May 24, 2013. Thus, her continuing problems in my view are predominantly if not entirely related to the slip and fall accident of May 24, 2013.

[69] Thus, Dr. Marshall, an orthopaedic specialist and the worker's treating surgeon, opined that the worker's lateral meniscus tear likely resulted from her workplace accident, given the mechanism as described above. Dr. Marshall also opines on the OA component of the worker's knee, which I will address later.

[70] The employer commissioned a review by Dr. Offierski, an orthopaedic surgeon. Although he noted the worker's age incorrectly (39 vs. 49), he understood the accident mechanism to be the one I have accepted above, namely "a twisting injury to the knee when she slipped and twisted the knee." Unlike Dr. Marshall, however, Dr. Offierski opined that the "symptoms arising from this injury were confined to the medial portion of the knee only." He further opined:

In summary, it is my opinion that the worker sustained a mild to moderate twisting injury to the right knee that could have torn the medial meniscus and required the arthroscopy and meniscectomy. The expected outcome of the above-noted surgery was that the worker would achieve relief of pain and disability 8 weeks after the surgery.

[71] Essentially, Dr. Offierski did not think that the injuring mechanism could have resulted in the lateral meniscus tear, and believed the medical evidence on file supported this opinion. Ms. Stewart submitted that Dr. Marshall was biased and as such less weight should be attributed to his opinion.

[72] I first note that what is before me on this matter is competing opinions from a doctor who treated the worker personally (and may therefore run the risk of becoming an advocate on her behalf) versus an opinion from a doctor hired by the accident employer who only completed a paper review and did not personally examine the worker. The Benefit of Doubt policy set out above notes that when evidence for or against is relatively equal, the matter is settled in favour of the worker. It would appear to me that, in this case, given the competing claims by two orthopaedic specialists, this policy may be applicable in tipping the weight towards the opinion of Dr. Marshall.

[73] In addition to this, however, I do note that prior to the workplace accident the worker had performed her regular duties for many years without issue, and although the medical evidence suggests that the worker had pre-existing OA, there is no evidence of significance suggesting that her right knee had in any way been symptomatic following surgery in 2001 (or 2003, as mentioned in some documentary evidence). Following the accident the worker was not able to return to her regular duties and her right knee remained symptomatic. Although the initial medical evidence on file indicated the worker had an issue with her kneecap and medial meniscus, this opinion was given prior to diagnostic testing.

[74] Therefore, given the opinion of Dr. Marshall and allowing his opinion the Benefit of Doubt over the competing opinion of Dr. Offierski, together with aforementioned medical evidence, and the fact that the worker's knee was asymptomatic prior to the injury but not thereafter, I find on the balance of probabilities that the worker has initial entitlement to a tear of her right lateral meniscus as a result of her workplace accident.

[75] Given that I have found that the lateral meniscus tear was properly allowed as part of this claim, the surgery to repair same is in order. As the worker suffered a loss of earnings from the date of surgery to October 13, 2013 she was also properly entitled to full LOE benefits for this period, as her loss of earnings was a result of her compensable condition.

[76] The worker's right knee has never recovered to its asymptomatic pre-accident condition, and as such she was awarded a NEL benefit for her torn right medial meniscus, and torn right lateral meniscus (as well as for post-traumatic OA). As the worker has entitlement for these injuries, and has an ongoing and permanent impairment, the worker accordingly has ongoing entitlement for the sequelae from these two torn menisci.

(b) Entitlement to post-traumatic OA

[77] The employer objected to entitlement for post-traumatic OA on the basis that the medical evidence, the time passed before diagnosis, and the medical literature, do not support the appearance of post-traumatic OA in this case. Although the Board specifically noted that any pre-existing OA was not compensable, at some point the worker was allowed entitlement to post traumatic OA, as ultimately reflected in her NEL award.

[78] I note the following documentary evidence from the Record:

- Memo #33, dated September 5, 2013 which contained the decision to allow eligibility for FLAP tear and surgery to correct same. It also noted no eligibility for OA as “condition was present prior to the date of accident and not caused by the accident.”

- A report dated October 20, 2014 from Dr. Marshall noted:

It should be noted that prior to her work -related injury, she was completely asymptomatic. The meniscal tear and posttraumatic arthritic changes are entirely in keeping with her injury. Although she has recovered from the meniscal symptoms, she continues to have significantly disabling posttraumatic arthritic symptoms, most notably in the lateral compartment. In the absence of proceeding with PRP injections, it is likely that she will require arthroscopy and microfracture of her lateral femoral condyle.

- Memo #145, dated November 17, 2014 which contained a review and opinion of Board Medical Consultant (BMC) Dr. Gelman. He noted:

[The worker] is a 49 year old woman who injured her right knee in May, 2013. This resulted in a meniscal tears and posttraumatic arthritis.

- Memo #188, dated November 23, 2015 which set out the consideration of a permanent impairment (PI) for the worker. It noted:

PI Recommendation:

Permanent work-related impairment appears evident for [the worker’s] right knee with diagnosis of torn right medial and lateral meniscus and post-traumatic arthritis.

[79] Thus, although OA was specifically disallowed at the outset of the claim, at some point the worker’s entitlement was expanded to include post-traumatic OA. From a review of the file, it appears as if the first mention of post-traumatic OA was found in Dr. Marshall’s October 20, 2014 report, which was then noted by BMC Dr. Gelman in his November 17, 2014 report. Roughly a year later the worker was granted a PI for the post-traumatic OA. The ARO and the NEL assessor noted that the worker had moderate pre-existing OA and discounted the final award to reflect this.

[80] Ms. Stewart submitted that the worker’s post-traumatic OA was most likely a result of her 2001 knee surgery, as the Tribunal’s Medical Discussion paper notes that the appearance of post-traumatic OA on imaging reports usually appears 10-20 years following an injury. She submitted that post-traumatic OA four months after an injury is not likely.

[81] I note, however, that the Discussions paper’s opinion on the appearance of post-traumatic OA states:

Meniscal injuries are mainly noted in the knee joint, but if one considers the glenoid and acetabular labrum, may also include the shoulder and hip. These cartilaginous structures have no blood supply except at their peripheral attachment, therefore in substantive tears, the meniscus cannot heal. Peripheral tears along the synovial marginal attachment may heal if reattached surgically. Removal of a meniscus causes late secondary arthritis even in cases of incomplete removal such as a flap tear. The radiographic appearance of the joint is altered in almost every case at 10-20 years and clinical symptoms may ensue.

[82] The report also states:

Clearly, a specific traumatic work related injury can lead to post traumatic OA, either by direct or indirect means.

[83] The report also noted:

Cartilage damage. Articular cartilage may be impacted without an early radiographic abnormality noted. This may occur in high impact ligamentous disruption especially in the knee; and by major compression injury in the hip. This can be seen on early MRI if available or, should the joint be operated upon either arthroscopically or by open technique. These injuries are not infrequent. These injuries may eventually lead to radiographic or clinical OA; whether it becomes clinically important will depend on many other factors as discussed.

[84] The evidence before me regarding the worker's right knee was that she had fluid drained from the knee in 2001 or 2003. Fluid on the knee can be the result of various issues ranging from injury to OA to obesity. It is noted that the worker did have previous issues with obesity, resulting in a gastric bypass in 2008. The worker rejected a request to have her medical records contemporaneous to this time produced, leaving me with the medical evidence referred to herein from her doctors, Dr. Offierski, and the Medical Discussion paper.

[85] I note that Ms. Stewart stated in her March 7, 2017 written submissions that the employer was prepared to accept "some minor post-traumatic osteoarthritis directly associated with the surgical repair to the medial meniscus."

[86] Unfortunately, the medical evidence before me is not definitive on where the worker's post-traumatic OA is most evident, and as I have found that the worker has entitlement to a lateral meniscus tear in addition to the medial meniscus tear, it would appear that any post-traumatic OA related to either meniscus would be the responsibility of the claim.

[87] I do note, in relation to Ms. Stewart's submission that the employer should not be responsible for the grade IV chondrolysis and grade II cartilage changes over the femoral condyle, that these conditions appear to be more properly categorized as part of the worker's pre-existing OA, and not as part of her post-traumatic OA. As noted above in the Medical Discussion paper, these changes would require the passage of some time in order for them to become as pronounced as they were so soon after the worker's accident. I will address the worker's pre-existing OA below.

[88] Thus, Dr. Marshall opined that the worker had some post-traumatic OA, an opinion that was repeated by BMC Dr. Gelman. Ms. Stewart submitted that some post-traumatic OA could be expected, and the medical literature suggests the same. Therefore, on the balance of probabilities, and with respect to the foregoing analysis, I find that the worker is entitled to recognition of post-traumatic OA as a result of her workplace injury.

(c) SIEF relief

[89] Ms. Stewart submitted that the employer was entitled to 90% SIEF relief on the basis that the worker had a major pre-existing condition with a minor accident. The Board found that the worker had a moderate pre-existing condition combined with a moderate accident leading to a 50% SIEF award.

[90] I will turn first to the extent of the pre-existing condition.

[91] Included in the Case Record were two Tribunal Medical Discussion Papers. The first, *Knee Conditions and Disability*, by Drs. Cameron and Tile, noted the following:

Difficult problems to resolve

A. Pre-existing arthritis

The injured person may have pre-existing arthritis of the knee, and because it develops slowly, they are not aware of it until an injury is superimposed on it. The resulting disability is often greater than one would expect for the amount of trauma. These people will often lose significant muscle strength which affects their knee stability and ability to recover. A particularly difficult problem is the person who has mild symptoms of osteoarthritis and then suffers a meniscal tear. Many of these people do poorly with arthroscopy due to the relative significance of the arthritis. The decision to consider arthroscopic surgery may be difficult. If the surgery results in considerable swelling and muscle atrophy, the return to pre-injury status is often dependant on the resolution of swelling and the recovery of muscle strength.

[92] This passage addresses the difficulty of resolving this issue, as well as noting that a person with OA and a meniscal tear do poorly after surgery due to the relative significance of the OA. This would appear to be similar to the case before me. There is no question the worker did have pre-existing OA, a meniscal tear (or two, as I have now found), and surgery from which she did not recover as expected.

[93] I note the following medical evidence on file:

- An MRI of the right knee dated July 16, 2013 which found:
 - There is mild medial and lateral compartment osteoarthritis.
 - No abnormal joint effusion. There is a small Baker's cyst.
- A report dated February 24, 2014 from Dr. Marshall which noted:
 - I am concerned that she may require reconstructive surgery for her advanced lateral compartment osteoarthritis.
- A report dated February 24, 2014 from Dr. Bhatia, a Pain specialist, which noted:
 - The last MRI, which was performed on 16 July 2013, revealed evidence of early -to-moderate osteoarthritis in the right knee along with tear of the medial and lateral menisci.
- A report dated March 24, 2014 from Dr. Marshall which noted:
 - Her recovery has been slow and she continues to have significant pain and significant underlying osteoarthritis. It is too soon to render a long-term prognosis.
- A report dated May 26, 2014 from Dr. Marshall which noted:
 - It should be noted that she had full thickness loss of articular cartilage on her lateral femoral condyle at time of arthroscopy.
- A report dated June 17, 2014 from Dr. Mahomed, an orthopaedic surgeon, which noted:
 - Arthroscopic findings at the time of surgery by Dr. Marshall would suggest that she has advanced lateral compartment OA, although this is not evidenced by her pain imaging.
- An MRI dated July 5, 2014 which noted:
 - Conclusion: Postsurgical changes of the medial meniscus. Combination of postsurgical changes of the lateral meniscus with an area of undersurface tearing of the anterior horn of the lateral meniscus. Severe chondrosis of the mid to posterior aspect of the lateral femoral condyle with milder changes involving the posterior aspect of the lateral tibial plateau. Mild patellofemoral chondrosis. Joint effusion with synovitis.
- A report dated August 11, 2014 from Dr. Marshall which noted:
 - It should be noted that her surgical findings are in keeping with her having had a degree of pre-existing injury to the lateral compartment, but this was significantly exacerbated

by the compensable traumatic injury, which resulted in a lateral meniscal tear and significant loss of articular cartilage over the lateral femoral condyle. She had no pre-existing symptoms prior to her injury.

- A report dated January 15, 2015 from Dr. Marshall which noted:

To clarify the findings at surgery, the subject had had a previous arthroscopy of her right knee in 2003. Subsequent to the arthroscopy she had been asymptomatic until her accident of May 24, 2013. At the time of her accident she had a slip and fall in which she violently rotated her right knee and landed on the lateral aspect. At time of arthroscopy, we found a complex tear of her lateral meniscus as well as full thickness loss of articular cartilage on the lateral femoral condyle and to a lesser extent the lateral tibial plateau. There was no degenerative change in the patellofemoral joint and very minimal change in the medial compartment. There was a small degenerative medial meniscal tear. Thus, although there may have been an element of some relatively mild asymptomatic degree of pre-existing osteoarthritis in the right knee, the findings at surgery of a large lateral meniscal tear and full thickness loss of articular cartilage and lateral femoral condyle were fully in keeping with the degree of violence that she suffered in the slip and fall accident of May 24, 2013. Thus, her continuing problems in my view are predominantly if not entirely related to the slip and fall accident of May 24, 2013.

[94] Dr. Offierski opined:

It is my opinion that the ongoing pain, swelling and disability in the right knee are due to the pre-existing arthritic condition of the lateral compartment of the right knee. The worker had previous arthroscopic surgery to the right knee fifteen years prior. It has been noted in the file that the worker has had problems with weight control over the years. This would account for the presence of the pre-existing arthritic condition in the lateral compartment of the knee.

...

I would disagree with the SIEF decision that states the reports do not indicate any pre-existing factors delaying the worker's recovery. The MRI scan and the operative findings document significant arthritic deterioration in the lateral compartment of the knee which has been determined to be the cause of the worker's ongoing disability.

In summary, it is my opinion that the worker sustained a mild to moderate twisting injury to the right knee that could have torn the medial meniscus and required the arthroscopy and meniscectomy. The expected outcome of the above-noted surgery was that the worker would achieve relief of pain and disability 8 weeks after the surgery.

[95] Thus, the foregoing medical evidence would suggest that the worker's medial compartment in the right knee showed signs of mild OA, while her lateral compartment showed signs of significant OA. Dr. Marshall noted in his February 24, 2014 report that her OA was "advanced," and then in his March 24, 2014 report that the OA was "significant." In his May 26, 2014 report he noted that the worker had a full thickness loss of her articular cartilage at the time of the surgery, which was echoed by Dr. Mohamed in his June 17, 2014 report and noted as "advanced" OA, although this was not reflected in some of the diagnostic imaging. However, in the July 5, 2014 MRI the worker's OA was noted as "severe." Dr. Offierski attributes the worker's OA to her previous right knee surgery in 2001 or 2003.

[96] The Tribunal Medical Discussion paper on Knee disability referenced above notes:

The symptoms may increase with a superimposed injury, (i.e. a meniscal tear with pre-existing osteoarthritis). In these cases the injury did not cause the osteoarthritis but the symptoms were precipitated by the injury and may be persistent because of the underlying osteoarthritis.

- [97] This would appear to be the case herein, where the worker has a superimposed injury on pre-existing OA. As noted, this indicates that the injury did not cause the OA, but the symptoms were caused by the injury and prolonged due to the OA.
- [98] Grade IV chondrolysis and Grade II cartilage changes are noted in the medical literature as OA by definition. According to The Steadman Clinic, a highly respected medical research and treatment centre:
- Although symptoms may not appear until later in life, articular cartilage problems are very common. Painful osteoarthritis develops when this smooth, gliding surface on the end of the bone has lost its cushioning, deformity develops, and bone rubs on bone. Damage may occur as the result of a sudden injury or wear and tear over many years. There are some people with damaged articular cartilage who display few symptoms and may not develop osteoarthritis until they are elderly.
- The development of osteoarthritis depends on several factors:
- The patient's age when the degeneration starts
- The patient's activity level and weight
- The presence of ligament damage
- Articular cartilage problems can be particularly difficult to treat because the onset, while occasionally sudden, often occurs gradually and thus is not immediately detected.
- What is an articular cartilage injury?
- An articular cartilage injury, or chondral injury, may occur as a result of a pivot or twist on a bent knee, similar to the motion that can cause a meniscus tear. Damage may also be the result of a direct blow to the knee. Chondral injuries may accompany an injury to a ligament, such as the anterior cruciate ligament. Small pieces of the articular cartilage can actually break off and float around in the knee as loose bodies, causing locking, catching, and/or swelling. More often, there is no clear history of a single injury. The patient's condition may, in fact, result from a series of minor injuries that have occurred over time. Articular cartilage also wears down as a person ages.
- Chondral damage is graded from mild to severe, and all grades can have characteristics of osteoarthritis.
- Grade IV - The cartilage may wear away completely, leaving the underlying bone exposed in small or widespread areas. When the involved areas are large, pain usually becomes more severe, causing a limitation in activity.
- [99] Thus, the worker's compensable accident was of the type that could lead to this type of injury, and subsequent OA. It is also, however, a degenerative type OA that could exist asymptotically prior to becoming symptomatic either on its own as a natural progression, or as the result of an injury. I note in particular that a Grade IV rating indicates that the cartilage has worn away completely, leaving the underlying bone exposed. This would appear to suggest that a period of time would have to pass before the articular cartilage progressed to a Grade IV level. Dr. Marshall noted that at the time of surgery the worker was already suffering from this Grade, which was just over four months after the workplace accident. It does not seem likely, given the medical opinion above, and by Dr. Offierski, that the worker could be suffering from such an advanced condition in such a short period of time. Rather, it would appear more likely that the worker had an asymptomatic condition which was rendered symptomatic by her workplace accident. As noted above, the condition can progress without symptoms over a number of years, and the worker does have risk factors given her age and prior issues with weight. Given the foregoing, it appears reasonable that the mechanism of the worker's accident (twisting) triggered

her previously asymptomatic degenerative condition. The doctor's findings above of pre-existing OA that was "significant," "advanced," and "severe" would suggest that the worker's pre-existing OA was more than moderate.

[100] Therefore, on the balance of probabilities and with a view to the foregoing evidence and analysis, I find that the worker had a major pre-existing OA condition in the lateral compartment of her knee, including Grade IV chondrolysis and Grade II cartilage changes over the lateral femoral condyle.

[101] The next step in determining SIEF relief is the severity of the accident. The Board found that the accident was of moderate severity, while the employer has contended that the accident was minor.

[102] The policy concerning SIEF sets out the definitions of the "Severity of Accident." It notes that a minor accident is one that is expected to cause a non-disabling or minor disabling injury; while a moderate accident is one that is expected to cause a disabling injury.

[103] Ms. Langan submitted two Tribunal decisions in support of her contention that the accident was of a moderate nature. *Decision No. 2275/12* concerns a worker who had a slip and fall on ice, during which he twisted his right knee and ankle. The Vice Chair in this decision set out the following:

Pursuant to Board policy, accident severity is determined in consideration of the extent to which the accident components (mechanics, position and environment) would be expected to cause disabling injury. Disability is elsewhere defined in the SIEF policy as a condition requiring treatment and disrupting employment. I agree with and adopt the comments of the Vice-Chair in *Decision No. 1404/11*.

According to Board policy, a minor accident is expected to cause non-disabling or minor disabling injury. I construe this to mean that a worker involved in a minor accident would be expected to return to work immediately, or at most, a day or two later. In my view, a moderate accident is one that would reasonably be expected to cause some disability in the form of several days or weeks off from work, or a period of modified work. The policy does not define the length of the expected disability associated with a moderate accident, although it is implied in the definitions of "moderate" and "major" that a moderate accident is not expected to cause permanent disability. This approach is consistent with the interpretation adopted in *Decision No. 529/07*.

In this appeal, the accident mechanics, position and environment include a fall from standing position, with twisting of the right knee and ankle, on icy terrain in winter. In my view, such an accident would be expected to cause an injury requiring health care and affecting the ability to work without restrictions, for more than a brief period of time. This finding is consistent with Tribunal cases which have found a "simple" slip and fall to be minor (for example, *Decisions No. 1732/12* and *2368/06*) but a fall complicated by other factors, such as uneven terrain or falling hard on the shoulder, to be moderate (for example, *Decisions No. 109/07* and *743/01*). Here, the worker's fall was complicated by the added element of twisting of the knee and ankle. While a permanent disability would not be expected in this case, neither would one expect only a minor disabling injury in light of the combination of the fall, the twisting, and the icy conditions. Several days or weeks of modified work, or time off work, would be a reasonably likely outcome of such a fall.

I therefore conclude that the severity of the accident was moderate.

[104] Ms. Stewart also submitted Tribunal decisions with regards to this issue. In *Decision No. 389/14* the Vice Chair referred to *Decision No. 1021/12* which "confirmed that the actual injuries

are not considered but rather the extent of disability the mechanics of the accident would reasonably be expected to cause.” In her conclusion, the Vice Chair set out what other decisions had found with respect to the severity of injury:

Decision Nos. 167/13 and 244/13 considered a slip and fall on ice to be an accident of moderate severity.

Decision No. 968/13 held that a slip and fall down a half dozen steps was an accident of major severity.

Decision No. 1181/13 considered a fall over a rolled carpet in a dimly lit hallway to be an accident of moderate severity.

Decision No. 2149/12 considered the case of a worker who slipped on a cardboard twisting her knee. It was held that the accident was minor in severity, in part because the worker did not fall to the ground.

A fall of a distance of two feet landing on the buttocks was considered to be an accident of moderate severity in *Decision No. 186/13*.

Decision No. 2403/12 held that a slip and fall while the worker was walking up a flight of stairs to be an accident of moderate severity.

As can be seen from the above, Tribunal decisions generally consider slips and falls to be of moderate severity if the worker actually falls to the ground rather than is injured by slipping but not falling. This is particularly true where stairs are involved: the decisions generally accept that the accident severity is moderate or major depending on the number of stairs involved. In this case, the number of stairs is unknown. In his report of injury (Form 6), the worker indicated that he “slipped on hardwood floor and landed on my back down the stairs.” In my view, such an accident can be expected to cause a disabling injury. I therefore conclude that the accident was moderate in severity.

[105] *Decision No. 1416/14* concerned a worker who had slipped on water on the floor and twisted his knee. The Vice Chair found that this was a common occurrence in a kitchen that generally resulted in little or no injury, and found the accident to be of minor severity.

[106] Thus, as noted by Vice Chair Martel above, Tribunal decisions generally consider slip and falls to be of moderate severity if the worker actually falls to the ground rather than an injury by slipping but not falling. In this case, the accepted mechanism of injury has the worker slipping and twisting her right knee, while falling to the ground and abrading her right hand.

[107] The documentary evidence on file, as well as the worker’s testimony, indicated that at the time of the accident she did not consider it to be of any significance, and that whatever injury had occurred was minor and would be better by Monday morning after a weekend of rest. This would appear to meet the minor accident severity definition as set out in Board policy, namely that the accident was expected to cause a non-disabling or minor disabling injury. I note, however, with regards to both the Decisions as set out above and Vice Chair Martel’s findings, that a fall combined with a slip could render an accident moderate in nature. Given the foregoing findings in prior Tribunal decisions, and that nature of an accident that resulted in the worker striking the ground with her right hand and knee, I find on the balance of probabilities that the severity of the accident was moderate in nature.

[108] As set out above, where the medical significance of a pre-existing condition is major in severity, and the accident was moderate in severity, the employer is entitled to 75% SIEF relief.

[109] Ms. Stewart submitted that should the employer's SIEF be increased, that it may be made retroactive to be reflected in the employer's NEER account. She submitted *Decision No. 7/16* in support of her position. The relevant portion of this decision noted:

As noted earlier, the employer has also requested that if increased SIEF relief is granted, that the Board be directed to retroactively adjust its NEER account to reflect that increase. *Decision No. 708/07* established that the Tribunal has jurisdiction to consider a request for a retroactive adjustment of the employer's NEER account in a SIEF appeal.

Since this claim had an accident date in 2010, the final NEER review took place on September 30, 2014. As Mr. Roberts noted in his submissions on this issue, the employer has been diligent in pursuing the issue of SIEF relief beginning in 2012 and it filed its Notice of Appeal to the Tribunal on August 22, 2014, prior to the final NEER review.

I accept that the employer has been diligent and persistent in pursuing entitlement to cost relief and note that a large component of the time it has taken to reach a final decision on this issue has been the result of the systemic delays within the WSIB and Tribunal appeals systems that are beyond the employer's control. As a result, I find it would be appropriate to retroactively adjust the employer's NEER account to reflect the increase in SIEF relief granted in this decision.

[110] In this instance, the NEER window would have closed in 2017 at which time the employer was actively and diligently seeking SIEF relief and an increase in same. As such, I find it would be appropriate to retroactively adjust the employer's NEER account to reflect the increase in SIEF relief granted in this decision.

DISPOSITION

[111] The appeal is allowed in part as follows:

1. The worker remains entitled to a right knee strain, right hand abrasions, right medial meniscus tear, right lateral meniscus tear, and post-traumatic OA.
2. The worker remains entitled to the surgery of October 4, 2013.
3. The worker remains entitled to LOE benefits from October 4, 2013 to October 13, 2013.
4. The employer is entitled to SIEF relief of 75%.
5. The Board is to retroactively adjust the employer's NEER account to reflect the increase in SIEF relief granted in this decision.

[112] The nature and duration of benefits flowing from this decision will be returned to the WSIB for further adjudication, subject to the usual rights of appeal.

DATED: March 26, 2018

SIGNED: K. Cooper